

TIDACT[®] GEL 1%

Ingredient(s):

Each gram contains:

Clindamycin Phosphate 11.9mg
(eq. to Clindamycin 10mg)

Preservatives:

Propyl Paraben 0.2mg
Methyl Paraben 1.8mg

Pharmacodynamics

1. Topical application of clindamycin reduces free fatty acid concentrations on the skin and suppresses the growth of *Propionibacterium acnes* (*Corynebacterium acnes*), an anaerobe found in sebaceous glands and follicles. *P. acnes* produces proteases, hyaluronidases, lipases, and chemotactic factors, all of which can produce inflammatory components or inflammation directly.

Pharmacokinetics

1. Clindamycin phosphate, although inactive *in vitro*, undergoes rapid *in vivo* hydrolysis to convert to the antibacterially active clindamycin.
2. Following multiple topical applications of clindamycin phosphate at a concentration equivalent to 10mg clindamycin/mL in an isopropyl alcohol and water solution, very low levels of clindamycin are present in the serum (0-3 ng/mL) and <0.2% of the dose is recovered in urine as clindamycin.
3. Clindamycin activity has been demonstrated in comedones from acne patients. The mean concentration of antibiotic activity in extracted comedones after 4 weeks of application was 597mcg per gram of comedonal material.
4. Clindamycin *in vitro* inhibits all *Propionibacterium acnes* cultures tested (MICs 0.4 mcg/mL).
5. Free fatty acids on the skin surface have been decreased approximately from 14% to 2% following application of clindamycin.

Indication(s):

Treatment of acne vulgaris.

Dosage and Administration:

Apply a thin film to the affected skin area twice daily.

Contraindication(s):

History of hypersensitivity to preparations containing clindamycin or lincomycin, regional enteritis or ulcerative colitis, or antibiotic-associated colitis. Its use in newborn is contraindicated.

Route of Administration: For external use only.

Precaution(s) / Warning(s):

Orally and parenterally administered clindamycin has been associated with severe colitis which may end fatally. Use of the topical formulation results in absorption of the antibiotic from the skin surface. Diarrhoea, bloody diarrhoea and colitis (including pseudomembranous colitis) have been reported with the use of topical and systemic clindamycin. Symptoms can occur after a few days, weeks or months following initiation of therapy. They have also been observed to begin up to several weeks after cessation of therapy. Studies indicate a toxin(s) produced by *Clostridium difficile* is one primary cause of antibiotic-associated colitis. The colitis is usually characterized by severe persistent diarrhoea and severe abdominal cramps and may be associated with the passage of blood and mucus. Endoscopic examination may reveal pseudomembranous colitis.

If significant or prolonged diarrhoea occurs, the drug should be discontinued. Large bowel endoscopy should be considered in cases of severe diarrhoea.

Antiperistaltic agents (eg. opiates and diphenoxylate with atropine) may prolong and/or worsen the condition.

Vancomycin has been found to be effective in the treatment of the antibiotic-associated pseudomembranous colitis produced by *Clostridium difficile*.

Mild cases of colitis may respond to discontinuance of clindamycin. Moderate to severe cases should be managed promptly with fluid, electrolyte and protein supplementation. Cholestyramine and colestipol resins have been shown to bind the toxin *in vitro*. If a resin is administered in conjunction with oral vancomycin, the medications should be administered several hours apart. Systemic corticoids and corticoid retention enemas may help relieve the colitis.

WARNING

- Clindamycin therapy has been associated with severe colitis which may end fatally.
- It should be reserved for serious infections where less toxic antimicrobial agents are inappropriate.
- It should not be used in patients with nonbacterial infections, such as most upper respiratory tract infections.
- Its use in newborns is contraindicated.

Interaction with Other Medicaments:

Antagonism has been demonstrated between clindamycin and erythromycin.

Pregnancy and Lactation:

Reproduction studies have been performed in rats and mice using SC and oral doses of clindamycin ranging from 100-600 mg/kg/day and have revealed no evidence of impaired fertility or harm to the fetus due to clindamycin. There are, however, no adequate and well-controlled studies in pregnant women. Because animal reproduction studies are not always predictive of human response, this drug should be used during pregnancy only if clearly needed.

It is not known whether clindamycin is excreted in human milk following use of Tidact gel. However, orally and parenterally administered clindamycin has been reported to appear in breast milk. As a general rule, nursing should not be undertaken while a patient is on a drug since many drugs are excreted in human milk.

Side Effect(s) / Adverse Reaction(s):

Clindamycin has been associated with severe colitis which may end fatally.

Cases of diarrhoea, bloody diarrhoea and colitis (including pseudomembranous colitis) have been reported as adverse reactions in patients treated with topical formulations of clindamycin.

Other effects which have been reported in association with the use of topical formulations of clindamycin include: Abdominal pain, contact dermatitis, gastrointestinal disturbances, gram-negative folliculitis, irritation, oily skin, sensitization, stinging of the eye.

Symptoms and Treatment for Overdosage, and Antidote(s):

Tidact gel contains an alcohol base which will cause burning and irritation of the eye. Bathe with cool tap water in the event of accidental contact with sensitive surfaces (eye, abraded skin, mucous membranes).

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Keep in an airtight container. Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):

A colourless transparent gel.

Aluminium tube of 10g, 15g, 20g and 30g.



Manufacturer and Product Registration Holder:
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